

What a defensible SSBCI determination file contains

Seven elements, each tied to the regulation that requires it. For the Medicare product, compliance and duals teams who now make this determination member by member.

WHY THIS CHANGED

VBID carried this book. Now SSBCI does.

15%

of D-SNP members were in
plans offering SSBCI in 2025

90%

are in 2026, following the
VBID sunset

Every one of those members carries an eligibility determination that has to be made on objective criteria, documented, and produced to CMS on request. The benefit design moved quickly. The documentation underneath it generally did not.

Source: Milliman, *State of the 2026 Medicare Advantage industry: Dual-eligible plan valuation and selected benefit offerings*. Figures are member weighted. The VBID model ended after plan year 2025.

What has to be in the record

1 The chronically ill determination

Record that the enrollee meets all three criteria: one or more comorbid and medically complex chronic conditions that is life threatening or significantly limits overall health or function, high risk of hospitalization or other adverse health outcomes, and a requirement for intensive care coordination.

42 CFR 422.102(f)(1)

2 The written policy you applied, and its version

Plans must have and apply written policies based on objective criteria. Record which version of the policy was in force on the date of the determination, not the version in force today.

42 CFR 422.102(f)(4)

3 The reasonable expectation basis

The item or service must have a reasonable expectation of improving or maintaining the health or overall function of the enrollee. Record the basis on which that expectation was formed.

42 CFR 422.102(f)(1)(ii)

4 The determination itself, dated

The decision, the date it was made, and the evidence relied on. A monthly roster is not a determination record.

42 CFR 422.102(f)(4)

The three most often missing

5 The enrollees you found ineligible

Plans must document each eligibility determination, whether eligible or ineligible. Most plans keep the approvals and lose the denials. The denial record is what shows the criteria were applied consistently.

42 CFR 422.102(f)(4)

6 Evidence of what you published, and when

The written policies and objective criteria must be listed on a public-facing website. Keep dated evidence of what was posted, since the obligation is to the criteria as published at the time.

42 CFR 422.102(f)(4)

7 Coverage-year stability and the retention clock

The objective criteria must be maintained without modification for the full coverage year. Supporting records sit inside a ten year retention obligation and are subject to HHS and Comptroller General inspection.

42 CFR 422.102(f)(4), 422.504(d) and (e)

THE TEST

Could you produce this, for any enrollee, on any date, in a week?

If the answer needs a vendor portal, an email thread, a care manager's notes and a spreadsheet, the record exists but it is not yet a file.

Anchor is the system of record for non-medical benefit execution. The plan sets eligibility policy and retains determination authority. Our team keeps the record.

[Talk to the team](#)